KAISER PERMANENTE

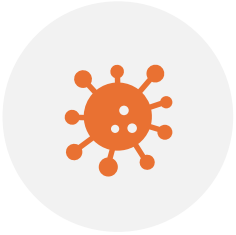
MAGNET RECOGNIZED
AMERICAN NURSES CREDENTIALING CENTER

SBC Infection Control & Prevention Updates

10/28/2026

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Agenda



HAI UPDATES



C.DIFF TESTING WORKFLOW



MTB REFRESHER



EID REFRESHER



CDPH WASTEWATER SURVEILLANCE

Days Since Last Healthcare-Associated Infection (HAI)					
C. *diff*	**Last Event Date**	**Today's Date**	**Number of Days**	**Location**	**C.DIFF Lessons Learned**
	8/1/2026	10/27/2026	**87**	4E	RCA completed
CAUTI	**Last Event Date**	**Today's Date**	**Number of Days**	**Location**	**CAUTI Lessons Learned**
	3/9/2026	10/27/2026	**232**	3W	RCA completed
CLABSI	**Last Event Date**	**Today's Date**	**Number of Days**	**Location**	**CLABSI Lessons Learned**
	3/11/2025	10/27/2026	**595**	ICU	RCA completed

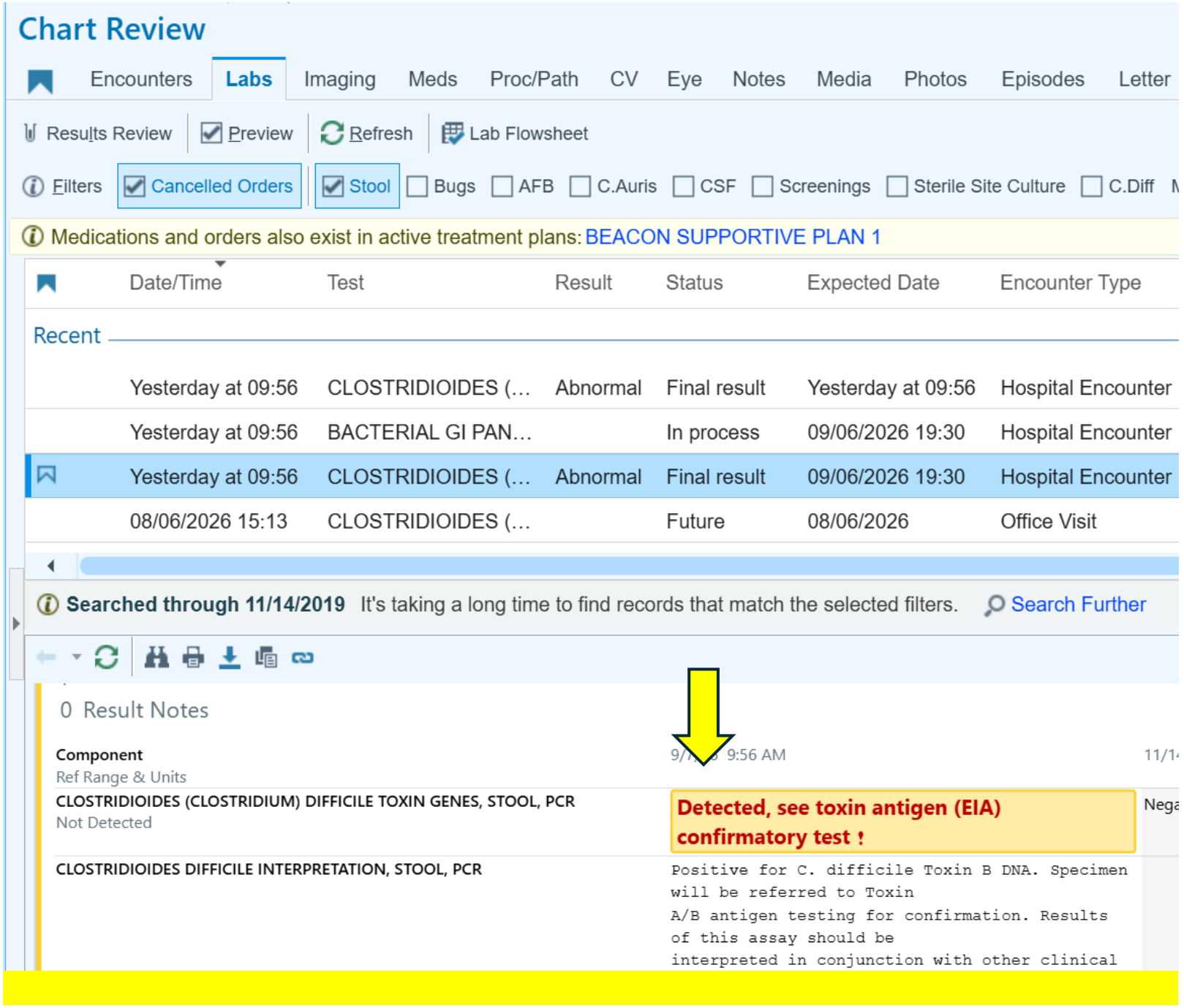
Days Since Last Healthcare-Associated Infection (HAI)					
C. *diff*	**Last Event Date**	**Today's Date**	**Number of Days**	**Location**	**C. *diff* Lessons Learned**
	6/14/2026	10/27/2026	135	6N	RCA Complete
CAUTI	**Last Event Date**	**Today's Date**	**Number of Days**	**Location**	**CAUTI Lessons Learned**
	10/15/2026	10/27/2026	12	6S	RCA In Progress
CLABSI	**Last Event Date**	**Today's Date**	**Number of Days**	**Location**	**CLABSI Lessons Learned**
	7/18/2026	10/27/2026	101	PICU	RCA Complete

Effective July 15, 2026, Kaiser Permanente will implement a new *Clostridioides difficile* (C. difficile) testing workflow. Testing will transition from the current antigen-first algorithm to a **PCR-first algorithm with reflex toxin A/B EIA testing** for PCR-positive specimens. This two-step approach is endorsed by the Infectious Diseases Society of America, American Society for Microbiology, and the American College of Gastroenterology as a preferred diagnostic strategy.

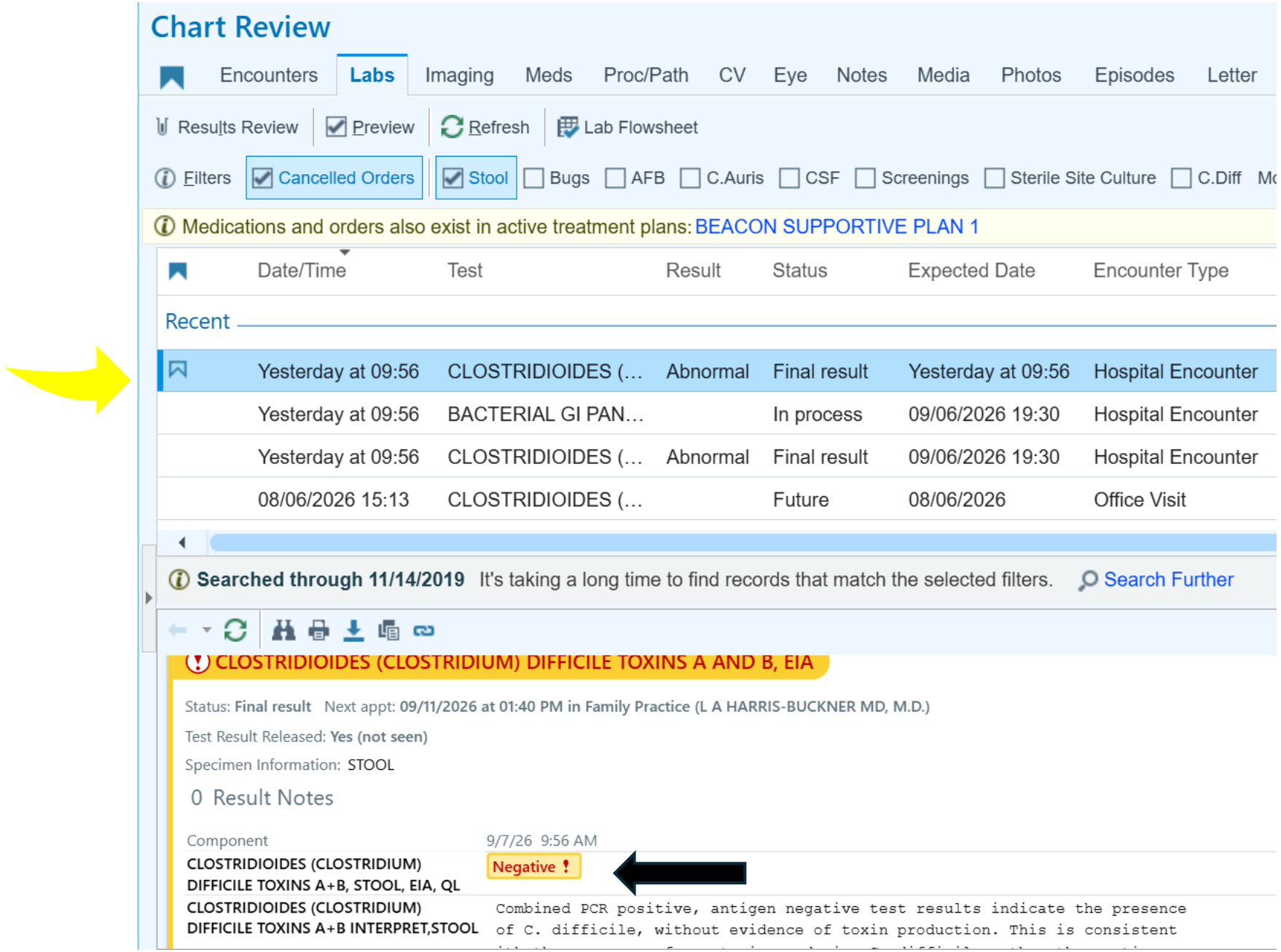
What Is Changing?

Result	Interpretation
PCR Negative	C. difficile not detected. No further testing needed. Repeat testing should not be performed for at least 7 days.
PCR Positive / Toxin Positive	Consistent with active C. difficile infection in a symptomatic patient.
PCR Positive / Toxin Negative	C. difficile DNA detected without evidence of active toxin production. This result may represent colonization rather than true infection. Clinical correlation is required — however, treatment should not be withheld when clinical suspicion for CDI is high.

1


Chart Review
Encounters
Labs
Imaging
Meds
Proc/Path
CV
Eye
Notes
Media
Photos
Episodes
Letter
Results Review
Preview
Refresh
Lab Flowsheet
Filters
Cancelled Orders
Stool
Bugs
AFB
C.Auris
CSF
Screenings
Sterile Site Culture
C.Diff
Medications and orders also exist in active treatment plans: BEACON SUPPORTIVE PLAN 1
Date/Time
Test
Result
Status
Expected Date
Encounter Type
Recent
Yesterday at 09:56 CLOSTRIDIOIDES (… Abnormal Final result Yesterday at 09:56 Hospital Encounter
Yesterday at 09:56 BACTERIAL GI PAN… In process 09/06/2026 19:30 Hospital Encounter
Yesterday at 09:56 CLOSTRIDIOIDES (… Abnormal Final result 09/06/2026 19:30 Hospital Encounter
08/06/2026 15:13 CLOSTRIDIOIDES (… Future 08/06/2026 Office Visit
Searched through 11/14/2019 It's taking a long time to find records that match the selected filters.
Search Further
0 Result Notes
Component
Ref Range & Units
9:56 AM
CLOSTRIDIOIDES (CLOSTRIDIUM) DIFFICILE TOXIN GENES, STOOL, PCR
Not Detected
Detected, see toxin antigen (EIA) confirmatory test !
CLOSTRIDIOIDES DIFFICILE INTERPRETATION, STOOL, PCR
Positive for C. difficile Toxin B DNA. Specimen will be referred to Toxin
A/B antigen testing for confirmation. Results of this assay should be
interpreted in conjunction with other clinical

2


Chart Review
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08/06/2026 15:13 CLOSTRIDIOIDES (… Future 08/06/2026 Office Visit
Searched through 11/14/2019 It's taking a long time to find records that match the selected filters.
Search Further
CLOSTRIDIOIDES (CLOSTRIDIUM) DIFFICILE TOXINS A AND B, EIA
Status: Final result Next appt: 09/11/2026 at 01:40 PM in Family Practice (L A HARRIS-BUCKNER MD, M.D.)
Test Result Released: Yes (not seen)
Specimen Information: STOOL
0 Result Notes
Component
9/7/26 9:56 AM
CLOSTRIDIOIDES (CLOSTRIDIUM) DIFFICILE TOXINS A+B, STOOL, EIA, QL
Negative !
CLOSTRIDIOIDES (CLOSTRIDIUM) DIFFICILE TOXINS A+B INTERPRET,STOOL
Combined PCR positive, antigen negative test results indicate the presence of C. difficile, without evidence of toxin production. This is consistent

New C.Diff Algorithm

Result	Interpretation
PCR Negative	C. difficile not detected. No further testing needed. Repeat test performed for at least 7 days.
PCR Positive / Toxin Positive	Consistent with active C. difficile infection in a symptomatic pa
PCR Positive / Toxin Negative	C. difficile DNA detected without evidence of active toxin produ represent colonization rather than true infection. Clinical correl however, treatment should not be withheld when clinical suspic

A positive PCR alone does not confirm active infection. Treatment decisions s incorporate toxin results and the overall clinical picture.

- Treatment decisions should incorporate toxin results and the overall clinical p



Adobe Acrobat Document

TB Cheat Sheet

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www.SBCounty.gov

SAN BERNARDINO COUNTY

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Health Officer

August 12, 2026

Health Advisory
Tuberculosis Screening and Treatment as Prevention
(Not Intended for Public Dissemination)
Key Messages

- San Bernardino County Department of Public Health (SBCDPH) is issuing this health advisory to provide guidance and recommendations to clinicians evaluating patients for tuberculosis (TB).
- Although TB disease (active TB) is preventable, TB incidence is increasing in San Bernardino County.
- **Assembly Bill 2132 (AB2132) requires primary care clinicians to screen all adult patients at least once for TB risk factor assessment.** If TB risk factors are identified, clinicians should offer blood test with interferon-gamma release assay (IGRA) or Purified Protein Derivative (PPD) skin testing as an alternative. IGRA should be offered to patients born outside the US.
- Active TB is preventable through early screening and timely treatment of TB infection/latent TB infection (LTBI).
- Preventing TB Disease in 4 Steps. LTBI Treatment Guidance

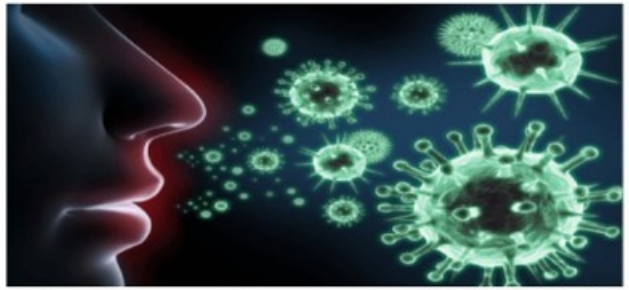
Background

- California's annual TB incidence was 5.4 cases per 100,000 persons; substantially higher than the national incidence rate of 3.0 per 100,000 in 2025.
- Active TB is preventable through early screening and timely treatment of latent TB.
- TB among children under five years old in California has increased. From 2020 to 2024, this age group had no more than 27 cases of TB each year. While in 2025, there were 35 new cases of TB among children under five years old.

Laboratory Studies	**Sputum Specimens (acid-fast bacilli AFB smear and culture)** • Specimens collected in 8-to-24-hour intervals • 1 early morning specimen 
Airborne Precautions	**Pulmonologist and/or Infectious Disease physician approval** is required *prior to* discontinuation of precautions 
Public Health	**Public health department approval** is required *prior to* discharge of suspected/confirmed patient with TB

2 MTB PCR test with 3 AFB sputum's need to be collected after patient is in an AIIR RM 11 & 33

RECENT TRAINING/EDUCATION:



Kaiser Permanente Los Angeles Medical Center

Special Pathogens/High Consequence Infectious Disease Preparedness Program
Clinical Skills Training

Date	Topic	Location
Wednesday 04/15/2026 9am & 1 pm	**Clinical Skills Day** IV, Foley, Speciman Collection, Inubatation, Central Line, IO, Level 1 Infuser	4733 Sunset Blvd, 1s Fl, Room 34

Kaiser Permanente Los Angeles Medical Center (LAMC) will be conducting a series of hands-on training sessions dedicated to Emerging Infectious Diseases.

Participants will Donn E.I.D. PPE and move through a series of skills sessions to become more comfortable should the situation ever present.

Registration:
https://www.eventbrite.com/e/emerging-infectious-disease-preparedness-program-clinical-skills-training-tickets-1985596345119?aff=oddtdtcreator

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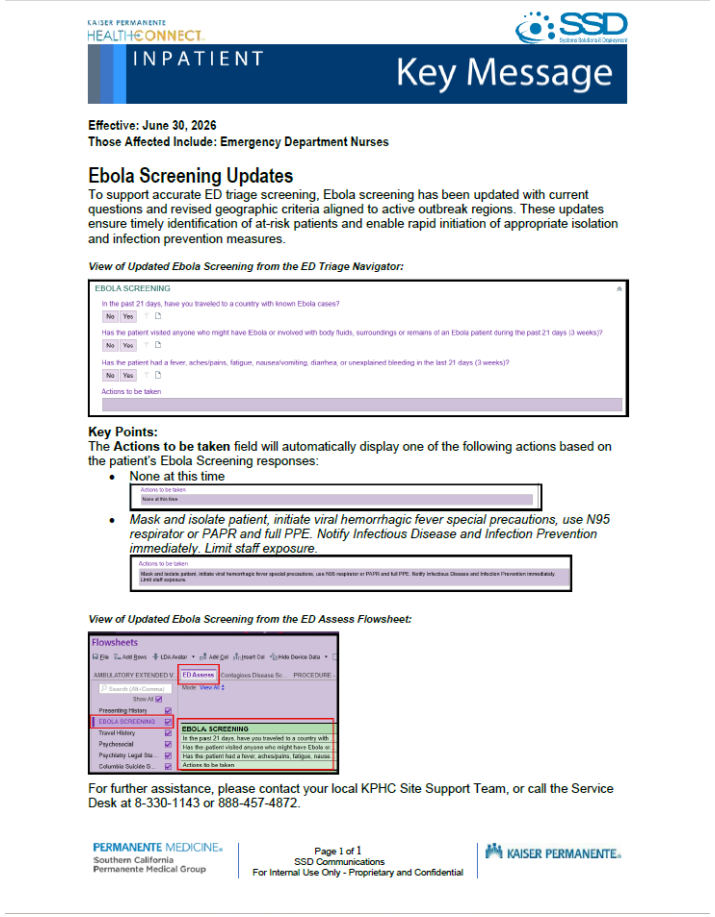






Emerging Infectious Disease

- Health Connect has updated the ED Ebola Screening Tool, to include revisions related to screening questions and current outbreak geography.
- If the screening is positive, staff must immediately:
  - Mask and isolate the patient
  - Initiate Viral Hemorrhagic Fever Special Precautions
  - Don appropriate PPE (N95 respirator or PAPR with full PPE)
  - Notify Infectious Diseases and Infection Prevention **immediately**
  - Limit staff exposure
- AIIR Rooms 10&33



KAISER PERMANENTE HEALTHCONNECT

SSD

INPATIENT **Key Message**

Effective: June 30, 2026
Those Affected Include: Emergency Department Nurses

Ebola Screening Updates

To support accurate ED triage screening, Ebola screening has been updated with current questions and revised geographic criteria aligned to active outbreak regions. These updates ensure timely identification of at-risk patients and enable rapid initiation of appropriate isolation and infection prevention measures.

View of Updated Ebola Screening from the ED Triage Navigator:

EBOLA SCREENING

In the past 21 days, have you traveled to a country with known Ebola cases?

No Yes

Has the patient visited anyone who might have Ebola or involved with body fluids, surroundings or remains of an Ebola patient during the past 21 days (3 weeks)?

No Yes

Has the patient had a fever, aches/pains, fatigue, nausea/vomiting, diarrhea, or unexplained bleeding in the last 21 days (3 weeks)?

No Yes

Actions to be taken

Key Points:
The **Actions to be taken** field will automatically display one of the following actions based on the patient's Ebola Screening responses:

- None at this time

Actions to be taken

None at this time

- *Mask and isolate patient, initiate viral hemorrhagic fever special precautions, use N95 respirator or PAPR and full PPE. Notify Infectious Disease and Infection Prevention immediately. Limit staff exposure.*

Actions to be taken

Mask and isolate patient, initiate viral hemorrhagic fever special precautions, use N95 respirator or PAPR and full PPE. Notify Infectious Disease and Infection Prevention immediately. Limit staff exposure.

View of Updated Ebola Screening from the ED Assess Flowsheet:

Flowsheets

EBOLA SCREENING

In the past 21 days, have you traveled to a country with...

Has the patient visited anyone who might have Ebola or...

Has the patient had a fever, aches/pains, fatigue, nausea...

Actions to be taken

For further assistance, please contact your local KPHC Site Support Team, or call the Service Desk at 8-330-1143 or 888-457-4872.

PERMANENTE MEDICINE
Southern California Permanente Medical Group

Page 1 of 1
SSD Communications
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California Wastewater Surveillance

California Wastewater Surveillance Dashboard

The last update for state and regional summaries was **10/23/2026** , with sewershed data and metrics being updated daily, Monday through Friday.

> Data from participating sites in the CDC NWSS commercial testing contract will be **temporarily unavailable** on the dashboard, while CDC transitions to a new contract. Historic data for these sites **remain accessible** . These sites have been **temporarily excluded** from state and regional summaries.

Statewide Overview (click the info box to be directed to the pathogen page)

SARS-CoV-2	Influenza A	Influenza B	RSV
Level: Low	**Level: Low**	**Level: Very Low**	**Level: Very Low**
Trend: Decrease	**Trend: Increase**	**Trend: Sporadic Detections**	**Trend: Sporadic Detections**

Each info box is colored to show the statewide virus level, as defined by the legend below.

Very High | High | Moderate | Low | Very Low | Not enough data